

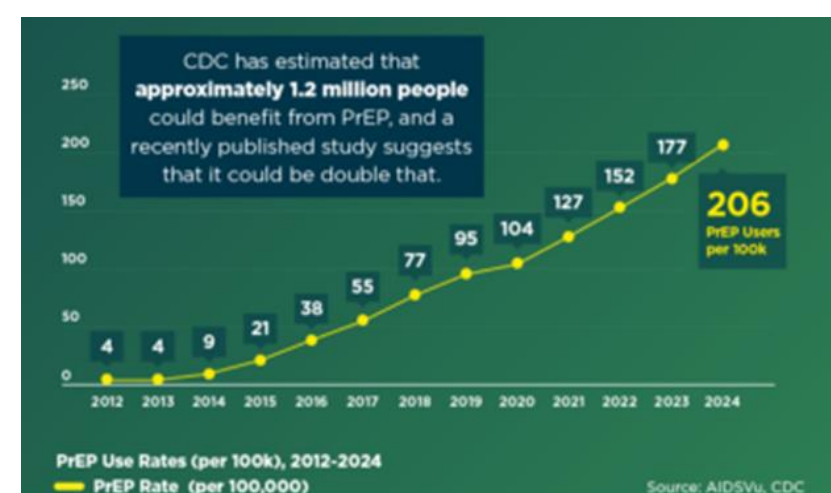
Development and implementation of a novel, pharmacist-led, long-acting HIV pre-exposure prophylaxis administration service in a health system outpatient pharmacy

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Background

Uptake of HIV Pre-Exposure Prophylaxis (PrEP)

- US far from goal of ending the HIV epidemic.
- PrEP lowers a person's risk of acquiring HIV.
- 2012-2022: States with highest PrEP had 38% *decrease* in new HIV diagnoses. States with lowest PrEP had 27% *increase*



Advances in HIV PrEP

- Highly effective PrEP available in long-acting (LA) injectables administered q 2-6 months.
 - Cabotegravir 600 mg IM q2 months (after load)
 - Lenacapavir 927 mg subcutaneously q6 mos (after load)
- LA-PrEP must be administered by healthcare provider
- Typically reimbursed through medical billing (CA Medicaid also covers through pharmacy benefit)

Context

- Primary care clinics may not have adequate staff, space, familiarity, or budget for LA-PrEP.
- SB 159 (2020) allows pharmacists to furnish HIV PEP and 60-day supply of HIV PrEP.
- SB 339 (2023) allows pharmacists to furnish ongoing PrEP if clinical conditions met & to receive payment for services through Medicaid.
- Both 159/339 generally reference "FDA-approved PrEP"

Program Description

UCSF Health provides LA-PrEP in the HIV Clinic. Providers at other UCSF locations expressed interest in providing LA-PrEP for their patients but did not have resources to establish a new service. The Ambulatory Pharmacy is establishing a long-acting injectable administration program to meet these needs.

Setting and Team



Setting: UCSF Health

- Integrated care system with medical centers and outpatient clinics
- Pharmacists may hold CPA or practice under state authorities
- Pharmacy enterprise has established specialty pharmacy & two new ambulatory pharmacies (10/2024)
- Pharmacy patient care clinic under med center license



Primary Team Members

- Pharmacy leadership: ACPO, Director Outpatient Pharmacy Services, Manager, Admin resident
- Pharmacy staff: Staff lead and all pharmacists
- Health IT: Clinical informatics pharmacist
- Subject matter expert: Faculty, School of Pharmacy
- Clinic champions: HIV clinic RN, Primary Care MD

Exploration (Oct 2024 - Jan 2025)

- Outer context:** Rationale for program development, determining needs/alternatives, identifying potential program champions, reviewing B&P codes and CA SB 159/339, discussing with legal
- Inner context:** Understanding UCSF priorities, exploring revenue streams, determining internal team & champions, operating as clinic

Preparation (Jan-May 2025)

- Communication/goals:** Weekly meetings to review progress/goals
- Enhancing pharmacist support:** SOPs, detailed workflow, training
- Enhancing Health IT infrastructure:** referrals, pharmacist privileges, visit type (INJ vs. IMM), Epic smartphrases, scheduling
- Purchasing materials:** Bed for consult room, other supplies
- Securing organizational approvals:** EMB, pharmacy leadership

Implementation (June 2025 -

- Obtaining product:** CAM workflow different than pharmacy acquisition
- Securing patient care space:** Shift to partnership with urgent care
- Separating screening visit & INJ appointment:** 340B/facility billing
- First patient 6/27/2025:** Onsite coaching with SME, debrief after

Sustainment (June 2025 -

- Ongoing training:** enhancing pharmacist confidence/experience
- Cycle for ordering supplies:** establish list/system/dates
- Following revenue cycle:** Pharmacist billing and drug reimbursement
- Discussing volume/capacity:** Interest growing (!), new medicines

Challenges During Pilot

- Drug procurement processes different for clinic & pharmacy
- 2-inch needles needed for larger BMI (>30)
- Staff pharmacists hold different privileges in EMR than ambulatory care pharmacists (e.g. facility fee billing)
- An existing drug order for the established (maintenance dose) patient – required new (pharmacist-led) order to operate fully under SB159/339
- Laboratory does not provide supplies for 3-site STI testing.
- Post administration billing required clarification



Lessons Learned To Date

- Creation of a pharmacy clinic within a community pharmacy is potential pathway for clinic-administered meds & medical billing
- Organization-specific interpretations of SB 159/339 as applied to long-acting injectable PrEP can be favorable
- Early assessment/testing of HIT infrastructure needed for "clinic-based" pharmacist vs. "pharmacy-based" pharmacist
- General SOP plus detailed step-by-step workflow should be created for optimal staff support
- Must procure drug/supplies not usually in pharmacy earlier
- Slow pilot is helpful for workflow, but info and patterns regarding revenue will be slow in returning
- A pharmacy-led injectable HIV PrEP service can support patient access and reduce clinic burden
- Planning requires active teamwork and facile communication across clinics, subject matter experts, health IT, medical center leadership, and pharmacy staff.

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